FOUNDATIONONE®CDx

PATIENT	TUMOR TYPE	REPORT DATE
Yang, Tzu Wei	Brain meningioma	30 May 2023
	COUNTRY CODE	ORDERED TEST #
	TW	ORD-1634767-01

ABOUT THE TEST FoundationOne®CDx is a next-generation sequencing (NGS) based assay that identifies genomic findings within hundreds of cancer-related genes.

PATIENT
DISEASE Brain meningioma
NAME Yang, Tzu Wei
DATE OF BIRTH 20 July 1953
SEX Male
MEDICAL RECORD # 24258568

PHYSICIAN
ORDERING PHYSICIAN Yeh, Yi-Chen
MEDICAL FACILITY Taipei Veterans General Hospital
ADDITIONAL RECIPIENT None
MEDICAL FACILITY ID 205872
PATHOLOGIST Not Provided

SPECIMEN
SPECIMEN SITE Brain
SPECIMEN ID S111-33716 C (PF23060)
SPECIMEN TYPE Slide Deck
DATE OF COLLECTION 30 August 2022
SPECIMEN RECEIVED 22 May 2023

Biomarker Findings

Microsatellite status - MS-Stable
Tumor Mutational Burden - 5 Muts/Mb

Genomic Findings

For a complete list of the genes assayed, please refer to the Appendix.

NF2 V50fs*73

Report Highlights

- Evidence-matched **clinical trial options** based on this patient's genomic findings: (p. 4)

BIOMARKER FINDINGS	THERAPY AND CLINICAL TRIAL IMPLICATIONS
Microsatellite status - MS-Stable	**No therapies or clinical trials.** See Biomarker Findings section
Tumor Mutational Burden - 5 Muts/Mb	**No therapies or clinical trials.** See Biomarker Findings section

GENOMIC FINDINGS	THERAPIES WITH CLINICAL RELEVANCE (IN PATIENT'S TUMOR TYPE)	THERAPIES WITH CLINICAL RELEVANCE (IN OTHER TUMOR TYPE)
NF2 - V50fs*73	none	none
6 Trials see p. 4		

NOTE Genomic alterations detected may be associated with activity of certain approved therapies; however, the agents listed in this report may have varied clinical evidence in the patient's tumor type. Therapies and the clinical trials listed in this report may not be complete and exhaustive. Neither the therapeutic agents nor the trials identified are ranked in order of potential or predicted efficacy for this patient, nor are they ranked in order of level of evidence for this patient's tumor type. This report should be regarded and used as a supplementary source of information and not as the single basis for the making of a therapy decision. All treatment decisions remain the full and final responsibility of the treating physician and physicians should refer to approved prescribing information for all therapies.

Therapies contained in this report may have been approved by the US FDA.

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BIOMARKER

Microsatellite status

RESULT
MS-Stable

POTENTIAL TREATMENT STRATEGIES

— Targeted Therapies —

On the basis of clinical evidence, MSS tumors are significantly less likely than MSI-H tumors to respond to anti-PD-1 immune checkpoint inhibitors[1-3], including approved therapies nivolumab and pembrolizumab[4]. In a retrospective analysis of 361 patients with solid tumors treated with pembrolizumab, 3% were MSI-H and experienced a significantly higher ORR compared with non-MSI-H cases (70% vs. 12%, p=0.001)[5].

FREQUENCY & PROGNOSIS

The reported incidence of MSI in meningioma has varied significantly between studies[6-12]. Published data investigating the prognostic implications of MSI in meningioma are limited (PubMed, Aug 2022).

FINDING SUMMARY

Microsatellite instability (MSI) is a condition of genetic hypermutability that generates excessive amounts of short insertion/deletion mutations in the genome; it generally occurs at microsatellite DNA sequences and is caused by a deficiency in DNA mismatch repair (MMR) in the tumor[13]. Defective MMR and consequent MSI occur as a result of genetic or epigenetic inactivation of one of the MMR pathway proteins, primarily MLH1, MSH2, MSH6, or PMS2[13-15]. This sample is microsatellite-stable (MSS), equivalent to the clinical definition of an MSS tumor: one with mutations in none of the tested microsatellite markers[16-18]. MSS status indicates MMR proficiency and typically correlates with intact expression of all MMR family proteins[13,15,17-18].

BIOMARKER

Tumor Mutational Burden

RESULT
5 Muts/Mb

POTENTIAL TREATMENT STRATEGIES

— Targeted Therapies —

On the basis of clinical evidence in solid tumors, increased TMB may be associated with greater sensitivity to immunotherapeutic agents, including anti-PD-L1[19-21], anti-PD-1 therapies[19-22], and combination nivolumab and ipilimumab[23-28]. In multiple pan-tumor studies, increased tissue tumor mutational burden (TMB) was associated with sensitivity to immune checkpoint inhibitors[19-22,29-33]. In the KEYNOTE 158 trial of pembrolizumab monotherapy for patients with solid tumors, significant improvement in ORR was observed for patients with TMB ≥10 Muts/Mb (as measured by this assay) compared with those with TMB <10 Muts/Mb in a large cohort that included multiple tumor types[29]; similar findings were observed in the KEYNOTE 028 and 012 trials[22]. At the same TMB cutpoint, retrospective analysis of patients with solid tumors treated with any checkpoint inhibitor identified that tissue TMB scores ≥ 10 Muts/Mb were associated with prolonged time to treatment failure compared with scores <10 muts/Mb (HR=0.68)[33]. For patients with solid tumors treated with nivolumab plus ipilimumab in the CheckMate 848 trial, improved responses were observed in patients with a tissue TMB ≥ 10 Muts/Mb independent of blood TMB at any cutpoint in matched samples[34]. However, support for higher TMB thresholds and efficacy was observed in the prospective Phase 2 MyPathway trial of atezolizumab for patients with pan-solid tumors, where improved ORR and DCR was seen in patients with TMB ≥ 16 Muts/Mb than those with TMB ≥ 10 and <16 Muts/Mb[32]. Similarly, analyses across several solid tumor types reported that patients with higher TMB (defined as ≥16-20 Muts/Mb) achieved greater clinical benefit from PD-1 or PD-L1-targeting monotherapy compared with patients with higher TMB treated with chemotherapy[35] or those with lower TMB treated with PD-1 or PD-L1-targeting agents[20].

FREQUENCY & PROGNOSIS

Meningioma harbors a median TMB of 1.8 mutations per megabase (muts/Mb), and 0.9% of cases have high TMB (>20 muts/Mb)[36]. Grade 2 or 3 meningiomas have been associated with a higher mutation rate than Grade 1 meningiomas; meningiomas were reported to have a smaller number of somatic alterations relative to other tumor types[37].

FINDING SUMMARY

Tumor mutation burden (TMB, also known as mutation load) is a measure of the number of somatic protein-coding base substitution and insertion/deletion mutations occurring in a tumor specimen. TMB is affected by a variety of causes, including exposure to mutagens such as ultraviolet light in melanoma[38-39] and cigarette smoke in lung cancer[40-41], treatment with temozolomide-based chemotherapy in glioma[42-43], mutations in the proofreading domains of DNA polymerases encoded by the POLE and POLD1 genes[44-48], and microsatellite instability (MSI)[44,47-48]. This sample harbors a TMB level associated with lower rates of clinical benefit from treatment with PD-1- or PD-L1-targeting immune checkpoint inhibitors compared with patients with tumors harboring higher TMB levels, based on several studies in multiple solid tumor types[20-21,29].

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GENE

NF2

ALTERATION
V50fs*73

HGVS VARIANT
NM_000268.3:c.148del (p.V50Cfs*73)

VARIANT CHROMOSOMAL POSITION
chr22:30032771-30032772

VARIANT ALLELE FREQUENCY (% VAF)
52.7%

POTENTIAL TREATMENT STRATEGIES

— Targeted Therapies —

Loss or inactivation of NF2 may also predict sensitivity to FAK inhibitors based on clinical data in mesothelioma[49] and meningioma[50] and strong preclinical data[51-53]. NF2 inactivating alterations may indicate sensitivity to mTOR inhibitors[54-57]. Two case studies reported clinical benefit for patients with NF2-mutated cancers, including urothelial carcinoma[58] and metaplastic breast cancer[59-60] treated with everolimus and temsirolimus, respectively. Whereas frequent adverse events precluded a recommended Phase 2 dose and schedule for the combination of the MEK inhibitor trametinib and the MTOR inhibitor everolimus in a Phase 1b trial for solid tumors[61], a retrospective study for patients with solid tumors who were heavily pretreated reported tolerable regimens of the combination for 74% (23/31) of patients, with 16 patients treated >3 months and evaluable patients achieving a median PFS of 6.5 months[62]. A Phase 1 trial of YAP/TEAD inhibitor VT3989 for patients with NF2-mutated tumors or mesothelioma reported 2 PRs for patients with heavily pretreated mesothelioma and 1 PR for a patient with heavily pretreated spindle cell sarcoma[63]. In a Phase 2 trial of the investigational FAK inhibitor GSK2256098 for patients with meningiomas harboring NF2 mutations, 10/12 (83%) patients with Grade 1 meningioma and 8/24 (33%) patients with Grade 2 or Grade 3 disease experienced PFS lasting ≥6 months; in the Grade 1 cohort, no objective responses were observed, but 10/12 (83%) patients achieved SD, while in the Grade 2 and Grade 3 cohort, 1/24 (4%) patients achieved a PR and 14/24 (58%) patients achieved SD[50].

FREQUENCY & PROGNOSIS

NF2 is the most commonly altered gene in meningioma, and patients with germline NF2 mutations are at increased risk for meningiomas, with the lifetime risk of meningioma estimated at up to 75%[64-65]. Loss of NF2 function via mutation or loss of expression has been reported in 40-60% of sporadic meningiomas[66-67]. NF2 mutation has been reported in 37% of meningioma samples analyzed in COSMIC (Sep 2022)[68]. One study reported that NF2 or chromosome 22 alterations occurred more frequently in high-grade (80%) vs low-grade (43%) meningioma samples analyzed[69]. NF2 mutation has been associated with monosomy 22 in sporadic meningioma, specifically for patients who are menopausal[70-71].

FINDING SUMMARY

Merlin, encoded by NF2, coordinates cell contact with growth signals; the inactivation of Merlin disrupts this mechanism and can lead to unrestrained growth despite cell contact[72]. Alterations such as seen here may disrupt NF2 function or expression[73-79].

POTENTIAL GERMLINE IMPLICATIONS

Heterozygous germline NF2 loss or inactivation is associated with neurofibromatosis type 2, which results in the development of vestibular schwannomas, meningiomas, ependymomas, and ocular disturbances[80-82]. Prevalence for this disorder in the general population is estimated to be 1:25,000[82]. In the appropriate clinical context, germline testing of NF2 is recommended.

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CLINICAL TRIALS

NOTE Clinical trials are ordered by gene and prioritized by: age range inclusion criteria for pediatric patients, proximity to ordering medical facility, later trial phase, and verification of trial information within the last two months. While every effort is made to ensure the accuracy of the information contained below, the information available in the public domain is continually updated and should be investigated by the physician or research staff. This is not a comprehensive list of all available clinical trials. Foundation Medicine displays a subset of trial options and ranks them in this order of descending priority: Qualification for pediatric trial → Geographical proximity → Later trial phase. Clinical trials listed here may have additional enrollment criteria that may require medical screening to determine final eligibility. For additional information about listed clinical trials or to conduct a search for additional trials, please see clinicaltrials.gov. Or, visit https://www.foundationmedicine.com/genomic-testing#support-services.

GENE
NF2

ALTERATION
V50fs*73

RATIONALE
Inactivation or loss of NF2 results in the dysregulation of mTOR and FAK pathway signaling. Therefore, mTOR and/or FAK inhibitors may be relevant for patients with NF2 inactivating mutations.

NCT03239015

PHASE 2

Efficacy and Safety of Targeted Precision Therapy in Refractory Tumor With Druggable Molecular Event

TARGETS
EGFR, ERBB4, ERBB2, PARP, mTOR, MET, ROS1, RET, VEGFRs, BRAF, CDK4, CDK6

LOCATIONS: Shanghai (China)

NCT04803318

PHASE 2

Trametinib Combined With Everolimus and Lenvatinib for Recurrent/Refractory Advanced Solid Tumors

TARGETS
mTOR, FGFRs, RET, PDGFRA, VEGFRs, KIT, MEK

LOCATIONS: Guangzhou (China)

NCT05125523

PHASE 1

A Study of Sirolimus for Injection (Albumin Bound) in Patients With Advanced Solid Tumors

TARGETS
mTOR

LOCATIONS: Tianjin (China)

NCT05036226

PHASE 1/2

COAST Therapy in Advanced Solid Tumors and Prostate Cancer

TARGETS
DDR2, ABL, SRC, KIT, mTOR

LOCATIONS: South Carolina

NCT01582191

PHASE 1

A Phase 1 Trial of Vandetanib (a Multi-kinase Inhibitor of EGFR, VEGFR and RET Inhibitor) in Combination With Everolimus (an mTOR Inhibitor) in Advanced Cancer

TARGETS
mTOR, EGFR, SRC, RET, VEGFRs

LOCATIONS: Texas

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NCT03203525

PHASE 1

Combination Chemotherapy and Bevacizumab With the NovoTTF-100L(P) System in Treating Participants With Advanced, Recurrent, or Refractory Hepatic Metastatic Cancer

TARGETS
VEGFA, mTOR

LOCATIONS: Texas

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NOTE One or more variants of unknown significance (VUS) were detected in this patient's tumor. These variants may not have been adequately characterized in the scientific literature at the time this report was issued, and/or the genomic context of these alterations makes their significance unclear. We choose to include them here in the event that they become clinically meaningful in the future.

ASXL1
NM_015338.5: c.395C>T
(p.S132L)
chr20:31016149

BRCA1
NM_007294.3: c.2083G>A
(p.D695N)
chr17:41245465

ERBB3
NM_001982.3: c.2900G>A
(p.R967K)
chr12:56493492

FGF6
NM_020996.1: c.562C>T
(p.R188W)
chr12:4543446

KMT2D (MLL2)
NM_003482.4: c.8054G>A
(p.R2685Q)
chr12:49433393

NOTCH2
NM_024408.3: c.4758A>C
(p.E1586D)
chr1:120466361

NOTCH3
NM_000435.2: c.4606A>G
(p.S1536G)
chr19:15285009

RET
NM_020975.4: c.874G>A
(p.V292M)
chr10:43601830

ROS1
NM_002944.2: c.6639C>A
(p.D2213E)
chr6:117622231 and
NM_002944.2: c.4058A>C
(p.Y1353S)
chr6:117677875

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FoundationOne CDx is designed to include genes known to be somatically altered in human solid tumors that are validated targets for therapy, either approved or in clinical trials, and/or that are unambiguous drivers of oncogenesis based on current knowledge. The current assay interrogates 324 genes as well as introns of 36 genes involved in rearrangements. The assay will be updated periodically to reflect new knowledge about cancer biology.

DNA GENE LIST: ENTIRE CODING SEQUENCE FOR THE DETECTION OF BASE SUBSTITUTIONS, INSERTION/DELETIONS, AND COPY NUMBER ALTERATIONS

ABL1	*ACVR1B*	*AKT1*	*AKT2*	*AKT3*	*ALK*	*ALOX12B*	*AMER1 (FAM123B or WTX)*	
APC	*AR*	*ARAF*	*ARFRP1*	*ARID1A*	*ASXL1*	*ATM*	*ATR*	*ATRX*
AURKA	*AURKB*	*AXIN1*	*AXL*	*BAP1*	*BARD1*	*BCL2*	*BCL2L1*	*BCL2L2*
BCL6	*BCOR*	*BCORL1*	*BRAF*	*BRCA1*	*BRCA2*	*BRD4*	*BRIP1*	*BTG1*
BTG2	*BTK*	*CALR*	*CARD11*	*CASP8*	*CBFB*	*CBL*	*CCND1*	*CCND2*
CCND3	*CCNE1*	*CD22*	*CD274 (PD-L1)*	*CD70*	*CD79A*	*CD79B*	*CDC73*	*CDH1*
CDK12	*CDK4*	*CDK6*	*CDK8*	*CDKN1A*	*CDKN1B*	*CDKN2A*	*CDKN2B*	*CDKN2C*
CEBPA	*CHEK1*	*CHEK2*	*CIC*	*CREBBP*	*CRKL*	*CSF1R*	*CSF3R*	*CTCF*
CTNNA1	*CTNNB1*	*CUL3*	*CUL4A*	*CXCR4*	*CYP17A1*	*DAXX*	*DDR1*	*DDR2*
DIS3	*DNMT3A*	*DOT1L*	*EED*	*EGFR*	*EMSY (C11orf30)*	*EP300*	*EPHA3*	*EPHB1*
EPHB4	*ERBB2*	*ERBB3*	*ERBB4*	*ERCC4*	*ERG*	*ERRFI1*	*ESR1*	*EZH2*
FANCA	*FANCC*	*FANCG*	*FANCL*	*FAS*	*FBXW7*	*FGF10*	*FGF12*	*FGF14*
FGF19	*FGF23*	*FGF3*	*FGF4*	*FGF6*	*FGFR1*	*FGFR2*	*FGFR3*	*FGFR4*
FH	*FLCN*	*FLT1*	*FLT3*	*FOXL2*	*FUBP1*	*GABRA6*	*GATA3*	*GATA4*
GATA6	*GID4 (C17orf39)*	*GNA11*	*GNA13*	*GNAQ*	*GNAS*	*GRM3*	*GSK3B*	*H3-3A (H3F3A)*
HDAC1	*HGF*	*HNF1A*	*HRAS*	*HSD3B1*	*ID3*	*IDH1*	*IDH2*	*IGF1R*
IKBKE	*IKZF1*	*INPP4B*	*IRF2*	*IRF4*	*IRS2*	*JAK1*	*JAK2*	*JAK3*
JUN	*KDM5A*	*KDM5C*	*KDM6A*	*KDR*	*KEAP1*	*KEL*	*KIT*	*KLHL6*
KMT2A (MLL)	*KMT2D (MLL2)*	*KRAS*	*LTK*	*LYN*	*MAF*	*MAP2K1 (MEK1)*	*MAP2K2 (MEK2)*	*MAP2K4*
MAP3K1	*MAP3K13*	*MAPK1*	*MCL1*	*MDM2*	*MDM4*	*MED12*	*MEF2B*	*MEN1*
MERTK	*MET*	*MITF*	*MKNK1*	*MLH1*	*MPL*	*MRE11 (MRE11A)*	*MSH2*	*MSH3*
MSH6	*MST1R*	*MTAP*	*MTOR*	*MUTYH*	*MYC*	*MYCL (MYCL1)*	*MYCN*	*MYD88*
NBN	*NF1*	*NF2*	*NFE2L2*	*NFKBIA*	*NKX2-1*	*NOTCH1*	*NOTCH2*	*NOTCH3*
NPM1	*NRAS*	*NSD2 (WHSC1 or MMSET)*		*NSD3 (WHSC1L1)*	*NT5C2*	*NTRK1*	*NTRK2*	*NTRK3*
P2RY8	*PALB2*	*PARP1*	*PARP2*	*PARP3*	*PAX5*	*PBRM1*	*PDCD1 (PD-1)*	*PDCD1LG2 (PD-L2)*
PDGFRA	*PDGFRB*	*PDK1*	*PIK3C2B*	*PIK3C2G*	*PIK3CA*	*PIK3CB*	*PIK3R1*	*PIM1*
PMS2	*POLD1*	*POLE*	*PPARG*	*PPP2R1A*	*PPP2R2A*	*PRDM1*	*PRKAR1A*	*PRKCI*
PRKN (PARK2)	*PTCH1*	*PTEN*	*PTPN11*	*PTPRO*	*QKI*	*RAC1*	*RAD21*	*RAD51*
RAD51B	*RAD51C*	*RAD51D*	*RAD52*	*RAD54L*	*RAF1*	*RARA*	*RB1*	*RBM10*
REL	*RET*	*RICTOR*	*RNF43*	*ROS1*	*RPTOR*	*SDHA*	*SDHB*	*SDHC*
SDHD	*SETD2*	*SF3B1*	*SGK1*	*SMAD2*	*SMAD4*	*SMARCA4*	*SMARCB1*	*SMO*
SNCAIP	*SOCS1*	*SOX2*	*SOX9*	*SPEN*	*SPOP*	*SRC*	*STAG2*	*STAT3*
STK11	*SUFU*	*SYK*	*TBX3*	*TEK*	*TENT5C (FAM46C)*		*TET2*	*TGFBR2*
TIPARP	*TNFAIP3*	*TNFRSF14*	*TP53*	*TSC1*	*TSC2*	*TYRO3*	*U2AF1*	*VEGFA*
VHL	*WT1*	*XPO1*	*XRCC2*	*ZNF217*	*ZNF703*			

DNA GENE LIST: FOR THE DETECTION OF SELECT REARRANGEMENTS

ALK	*BCL2*	*BCR*	*BRAF*	*BRCA1*	*BRCA2*	*CD74*	*EGFR*	*ETV4*
ETV5	*ETV6*	*EWSR1*	*EZR*	*FGFR1*	*FGFR2*	*FGFR3*	*KIT*	*KMT2A (MLL)*
MSH2	*MYB*	*MYC*	*NOTCH2*	*NTRK1*	*NTRK2*	*NUTM1*	*PDGFRA*	*RAF1*
RARA	*RET*	*ROS1*	*RSPO2*	*SDC4*	*SLC34A2*	*TERC**	*TERT***	*TMPRSS2*

*TERC is an NCRNA

**Promoter region of TERT is interrogated

ADDITIONAL ASSAYS: FOR THE DETECTION OF SELECT CANCER BIOMARKERS

Homologous Recombination status
Loss of Heterozygosity (LOH) score
Microsatellite (MS) status
Tumor Mutational Burden (TMB)

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ABOUT FOUNDATIONONE CDX

FoundationOne CDx was developed and its performance characteristics determined by Foundation Medicine, Inc. (Foundation Medicine). FoundationOne CDx may be used for clinical purposes and should not be regarded as purely investigational or for research only. Foundation Medicine's clinical reference laboratories are qualified to perform high-complexity clinical testing.

Please refer to technical information for performance specification details: www.rochefoundationmedicine.com/f1cdxtech.

INTENDED USE

FoundationOne®CDx (F1CDx) is a next generation sequencing based in vitro diagnostic device for detection of substitutions, insertion and deletion alterations (indels), and copy number alterations (CNAs) in 324 genes and select gene rearrangements, as well as genomic signatures including microsatellite instability (MSI), tumor mutational burden (TMB), and for selected forms of ovarian cancer, loss of heterozygosity (LOH) score, using DNA isolated from formalin-fixed, paraffin-embedded (FFPE) tumor tissue specimens. The test is intended as a companion diagnostic to identify patients who may benefit from treatment with therapies in accordance with approved therapeutic product labeling. Additionally, F1CDx is intended to provide tumor mutation profiling to be used by qualified health care professionals in accordance with professional guidelines in oncology for patients with solid malignant neoplasms.

TEST PRINCIPLE

FoundationOne CDx will be performed exclusively as a laboratory service using DNA extracted from formalin-fixed, paraffin-embedded (FFPE) tumor samples. The proposed assay will employ a single DNA extraction method from routine FFPE biopsy or surgical resection specimens, 50-1000 ng of which will undergo whole-genome shotgun library construction and hybridization-based capture of all coding exons from 309 cancer-related genes, one promoter region, one non-coding (ncRNA), and select intronic regions from 34 commonly rearranged genes, 21 of which also include the coding exons. The assay therefore includes detection of alterations in a total of 324 genes.

Using an Illumina® HiSeq platform, hybrid capture–selected libraries will be sequenced to high uniform depth (targeting >500X median coverage with >99% of exons at coverage >100X). Sequence data will be processed using a customized analysis pipeline designed to accurately detect all classes of genomic alterations, including base substitutions, indels, focal copy number amplifications, homozygous gene deletions, and selected genomic rearrangements (e.g.,gene fusions). Additionally, genomic signatures including loss of heterozygosity (LOH), microsatellite instability (MSI) and tumor mutational burden (TMB) will be reported.

THE REPORT

Incorporates analyses of peer-reviewed studies and other publicly available information identified by Foundation Medicine; these analyses and information may include associations between a molecular alteration (or lack of alteration) and one or more drugs with potential clinical benefit (or potential lack of clinical benefit), including drug candidates that are being studied in clinical research. The F1CDx report may be used as an aid to inform molecular eligibility for clinical trials. Note: A finding of biomarker alteration does not necessarily indicate pharmacologic effectiveness (or lack thereof) of any drug or treatment regimen; a finding of no biomarker alteration does not necessarily indicate lack of pharmacologic effectiveness (or effectiveness) of any drug or treatment regimen.

Diagnostic Significance

FoundationOne CDx identifies alterations to select cancer-associated genes or portions of genes (biomarkers). In some cases, the Report also highlights selected negative test results regarding biomarkers of clinical significance.

Qualified Alteration Calls (Equivocal and Subclonal)

An alteration denoted as "amplification – equivocal" implies that the FoundationOne CDx assay data provide some, but not unambiguous, evidence that the copy number of a gene exceeds the threshold for identifying copy number amplification. The threshold used in FoundationOne CDx for identifying a copy number amplification is four (4) for *ERBB2* and six (6) for all other genes. Conversely, an alteration denoted as "loss – equivocal" implies that the FoundationOne CDx assay data provide some, but not unambiguous, evidence for homozygous deletion of the gene in question. An alteration denoted as "subclonal" is one that the FoundationOne CDx analytical methodology has identified as being present in <10% of the assayed tumor DNA.

Ranking of Therapies and Clinical Trials

Ranking of Therapies in Summary Table
Therapies are ranked based on the following criteria: Therapies with clinical benefit (ranked alphabetically within each evidence category), followed by therapies associated with resistance (when applicable).

Ranking of Clinical Trials
Pediatric trial qualification → Geographical proximity → Later trial phase.

NATIONAL COMPREHENSIVE CANCER NETWORK® (NCCN®) CATEGORIZATION

Biomarker and genomic findings detected may be associated with certain entries within the NCCN Drugs & Biologics Compendium® (NCCN Compendium®) (www.nccn.org). The NCCN Categories of Evidence and Consensus indicated reflect the highest possible category for a given therapy in association with each biomarker or genomic finding. Please note, however, that the accuracy and applicability of these NCCN categories within a report may be impacted by the patient's clinical history, additional biomarker information, age, and/or co-occurring alterations. For additional information on the NCCN categories, please refer to the NCCN Compendium®. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines®). © National Comprehensive Cancer Network, Inc. 2023. All rights reserved. To view the most recent and complete version of the guidelines, go online to NCCN.org. NCCN makes no warranties of any kind whatsoever regarding their content, use or application and disclaims any responsibility for their application or use in any way.

Limitations

1. In the fraction-based MSI algorithm, a tumor specimen will be categorized as MSI-H, MSS, or MS-Equivocal according to the fraction of microsatellite loci determined to be altered or unstable (i.e., the fraction unstable loci score). In the F1CDx assay, MSI is evaluated based on a genome-wide analysis across >2000 microsatellite loci. For a given microsatellite locus, non-somatic alleles are discarded, and the microsatellite is categorized as unstable if remaining alleles differ from the reference genome. The final fraction unstable loci score is calculated as the number of unstable microsatellite loci divided by the number of evaluable microsatellite loci. The MSI-H and MSS cut-off thresholds were determined by

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analytical concordance to a PCR comparator assay using a pan-tumor FFPE tissue sample set. Patients with results categorized as "MS-Stable" with median exon coverage <300X, "MS-Equivocal," or "Cannot Be Determined" should receive confirmatory testing using a validated orthogonal (alternative) method.

2. TMB by F1CDx is determined by counting all synonymous and non-synonymous variants present at 5% allele frequency or greater (after filtering) and the total number is reported as mutations per megabase (mut/Mb) unit. Observed TMB is dependent on characteristics of the specific tumor focus tested for a patient (e.g., primary vs. metastatic, tumor content) and the testing platform used for the detection; therefore, observed TMB results may vary between different specimens for the same patient and between detection methodologies employed on the same sample. The TMB calculation may differ from TMB calculations used by other assays depending on variables such as the amount of genome interrogated, percentage of tumor, assay limit of detection (LoD), filtering of alterations included in the score, and the read depth and other bioinformatic test specifications. Refer to the SSED for a detailed description of these variables in FMI's TMB calculation https://www.accessdata.fda.gov/cdrh_docs/pdf17/P170019B.pdf. The clinical validity of TMB defined by this panel has been established for TMB as a qualitative output for a cut-off of 10 mutations per megabase but has not been established for TMB as a quantitative score.
3. Homologous Recombination status may be reported for epithelial ovarian, peritoneal, or Fallopian tube carcinomas (Coleman et al., 2017; 28916367). Samples with deleterious *BRCA1/2* alteration and/or Loss of Heterozygosity (LOH) score ≥ 16% will be reported as "HRD Positive" and samples with absence of these findings will be reported as "HRD Not Detected," agnostic of potential secondary *BRCA1/2* reversion alterations. Certain potentially deleterious missense or small in-frame deletions in *BRCA1/2* may not be classified as deleterious and, in the absence of an elevated LOH profile, samples with such mutations may be classified as "HRD Not Detected." A result of "HRD Not Detected" does not rule out the presence of a *BRCA1/2* alteration or an elevated LOH profile outside the assay performance characteristic limitations.
4. The LOH score is determined by analyzing SNPs spaced at 1Mb intervals across the genome on the FoundationOne CDx test and extrapolating an LOH profile, excluding arm- and chromosome-wide LOH segments. Detection of LOH has been verified only for ovarian cancer patients, and the LOH score result may be reported for epithelial ovarian, peritoneal, or Fallopian tube carcinomas. The LOH score will be reported as "Cannot Be Determined" if the sample is not of sufficient quality to confidently determine LOH. Performance of the LOH classification has not been established for samples below 35% tumor content. There may be potential interference of ethanol with LOH detection. The interfering effects of xylene, hemoglobin, and triglycerides on the LOH score have not been demonstrated.
5. Alterations reported may include somatic (not inherited) or germline (inherited) alterations; however, the test does not distinguish between germline and somatic alterations. The test does not provide information about susceptibility.
6. Biopsy may pose a risk to the patient when archival tissue is not available for use with the assay. The patient's physician should determine whether the patient is a candidate for biopsy.
7. Reflex testing to an alternative FDA approved companion diagnostic should be performed for patients who have an *ERBB2* amplification result detected with copy number equal to 4 (baseline ploidy of tumor +2) for confirmatory testing. While this result is considered negative by FoundationOne®CDx (F1CDx), in a clinical concordance study with an FDA approved FISH test, 70% (7 out of 10 samples) were positive, and 30% (3 out of 10 samples) were negative by the FISH test with an average ratio of 2.3. The frequency of *ERBB2* copy number 4 in breast cancer is estimated to be approximately 2%. Multiple references listed in https://www.mycancergenome.org/content/disease/breast-cancer/ERBB2/238/ report the frequency of HER2 overexpression as 20% in breast cancer. Based on the F1CDx HER2 CDx concordance study, approximately 10% of HER2 amplified samples had copy number 4. Thus, total frequency is conservatively estimated to be approximately 2%.

REPORT HIGHLIGHTS

The Report Highlights includes select genomic and therapeutic information with potential impact on patient care and treatment that is specific to the genomics and tumor type of the sample analyzed. This section may highlight information including targeted therapies with potential sensitivity or resistance; evidence-matched clinical trials; and variants with potential diagnostic, prognostic, nontargeted treatment, germline, or clonal hematopoiesis implications. Information included in the Report Highlights is expected to evolve with advances in scientific and clinical research. Findings included in the Report Highlights should be considered in the context of all other information in this report and other relevant patient information. Decisions on patient care and treatment are the responsibility of the treating physician.

VARIANT ALLELE FREQUENCY

Variant Allele Frequency (VAF) represents the fraction of sequencing reads in which the variant is observed. This attribute is not taken into account for therapy inclusion, clinical trial matching, or interpretive content. Caution is recommended in interpreting VAF to indicate the potential germline or somatic origin of an alteration, recognizing that tumor fraction and tumor ploidy of samples may vary.

Precision of VAF for base substitutions and indels

BASE SUBSTITUTIONS	%CV*
Repeatability	5.11 - 10.40
Reproducibility	5.95 - 12.31
INDELS	**%CV***
Repeatability	6.29 - 10.00
Reproducibility	7.33 - 11.71

**Interquartile Range = 1st Quartile to 3rd Quartile*

VARIANTS TO CONSIDER FOR FOLLOW-UP GERMLINE TESTING

The variants indicated for consideration of follow-up germline testing are 1) limited to reportable short variants with a protein effect listed in the ClinVar genomic database (Landrum et al., 2018; 29165669) as Pathogenic, Pathogenic/Likely Pathogenic, or Likely Pathogenic (by an expert panel or multiple submitters), 2) associated with hereditary cancer-predisposing disorder(s), 3) detected at an allele frequency of >10%, and 4) in select genes reported by the ESMO Precision Medicine Working Group (Mandelker et al., 2019; 31050713) to have a greater than 10% probability of germline origin if identified during tumor sequencing. The selected genes are *ATM, BAP1, BRCA1, BRCA2, BRIP1, CHEK2, FH, FLCN, MLH1, MSH2, MSH6, MUTYH, PALB2, PMS2, POLE, RAD51C, RAD51D, RET, SDHA, SDHB, SDHC, SDHD, TSC2,* and *VHL*, and are not inclusive of all cancer susceptibility genes. The content in this report should not substitute for genetic counseling or follow-up germline testing, which is needed to distinguish whether a finding in this patient's

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tumor sequencing is germline or somatic. Interpretation should be based on clinical context.

VARIANTS THAT MAY REPRESENT CLONAL HEMATOPOIESIS

Variants that may represent clonal hematopoiesis (CH) are limited to select reportable short variants in defined genes identified in solid tumors only. Variant selection was determined based on gene tumor-suppressor or oncogene status, known role in solid tumors versus hematological malignancies, and literature prevalence. The defined genes are *ASXL1, CBL, DNMT3A, IDH2, JAK2, KMT2D (MLL2), MPL, MYD88, SF3B1, TET2,* and *U2AF1* and are not inclusive of all CH genes. The content in this report should not substitute for dedicated hematological workup. Comprehensive genomic profiling of solid tumors detects nontumor alterations that are due to CH. Patient-matched peripheral blood mononuclear cell sequencing is required to conclusively determine if this alteration is present in tumor or is secondary to CH. Interpretation should be based on clinical context.

LEVEL OF EVIDENCE NOT PROVIDED

Drugs with potential clinical benefit (or potential lack of clinical benefit) are not evaluated for source or level of published evidence.

NO GUARANTEE OF CLINICAL BENEFIT

This Report makes no promises or guarantees that a particular drug will be effective in the treatment of disease in any patient. This Report also makes no promises or guarantees that a drug with potential lack of clinical benefit will in fact provide no clinical benefit.

NO GUARANTEE OF REIMBURSEMENT

Foundation Medicine makes no promises or guarantees that a healthcare provider, insurer or other third party payor, whether private or governmental, will reimburse a patient for the cost of FoundationOne CDx.

TREATMENT DECISIONS ARE RESPONSIBILITY OF PHYSICIAN

Drugs referenced in this Report may not be suitable for a particular patient. The selection of any, all or none of the drugs associated with potential clinical benefit (or potential lack of clinical benefit) resides entirely within the discretion of the treating physician. Indeed, the information in this Report must be considered in conjunction with all other relevant information regarding a particular patient, before the patient's treating physician recommends a course of treatment. Decisions on patient care and treatment must be based on the independent medical judgment of the treating physician, taking into consideration all applicable information concerning the patient's condition, such as patient and family history, physical examinations, information from other diagnostic tests, and patient preferences, in accordance with the standard of care in a given community. A treating physician's decisions should not be based on a single test, such as this Test, or the information contained in this Report. Certain sample or variant characteristics may result in reduced sensitivity. FoundationOne CDx is performed using DNA derived from tumor, and as such germline events may not be reported.

SELECT ABBREVIATIONS

ABBREVIATION	DEFINITION
CR	Complete response
DCR	Disease control rate
DNMT	DNA methyltransferase
HR	Hazard ratio
ITD	Internal tandem duplication
MMR	Mismatch repair
muts/Mb	Mutations per megabase
NOS	Not otherwise specified
ORR	Objective response rate
OS	Overall survival
PD	Progressive disease
PFS	Progression-free survival
PR	Partial response
SD	Stable disease
TKI	Tyrosine kinase inhibitor

REFERENCE SEQUENCE INFORMATION

Sequence data is mapped to the human genome, Genome Reference Consortium Human Build 37 (GRCh37), also known as hg19.

MR Suite Version (RG) 7.9.0

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The median exon coverage for this sample is 825x

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